Dr Harsh Vardhan and Smriti Irani, involved industry experts and large private players who then put together an SOP document in 24 hours. A pilot was run which involved local manufacturers as well and the South India Textile Research Association was looped in to test samples. However, on 22 March, it emerged that only four manufacturers passed the quality test. Thus, Operation PPE Coverall was launched by the Ministry of Textiles on 24 March under which a working business ecosystem where local manufacturers could be guided by bigger companies in the market or industry associations was enabled. In order to bridge the information gap, the Defence Research and Development Organisation (DRDO), the Alternative Energy Promotion Centre, the Bureau of Indian Standards (BIS) and the Confederation of Indian Industry were involved.[6] The efforts yielded great results, as in a matter of four–six weeks not only had India become self-sufficient in producing PPE kits, but we were beginning to export them. In another two and a half months, by July 2020, not only was our stockpile in surplus, but we had already exported about 23 lakh PPE kits to the US, Senegal, Slovenia, the UAE and the UK.

Similarly, as opposed to the dismal 3.35 lakh imported N95 masks which we had in reserve in March 2020, we strengthened our end-to-end manufacturing chain which led to us being able to produce 40 million N95 masks by July.[7]

While stories of achieving sufficiency in manufacturing of N95 masks and PPE kits are noteworthy, it is India's ventilator manufacturing story which establishes the true spirit of the Indian public–private entrepreneurial state model. In January 2020, India only had about 16,000 ventilators in public health facilities and another 29,000 in private care hospitals in circulation.[8] Most of these were imported from Europe and China at an average cost of about ₹15 lakh per unit. Domestic manufacturing of ventilators or its components was insignificant; as a matter of fact, neither there existed domestic standards for manufacturing of these units nor were they regulated medical devices with a BIS certification. Even the very few which were made in India were heavily dependent on importing components from various countries. In the forecast meeting at the PMO in mid-March, the team had projected India's requirement at about 60,000 pieces. In order to meet immediate demand, the Ministry of External Affairs approached China to supply about 10,000 ventilators at the earliest; however, China expressed its inability to supply since it was facing an acute